



Pediatric Symptom Checklist-17 (PSC-17)

<i>Client Name</i>	Generic Client	<i>Date administered</i>	3 Feb 2026
<i>Date of birth (age)</i>	14 Dec 2012 (13)	<i>Time taken</i>	41s
<i>Assessor</i>	Dr David Hegarty		

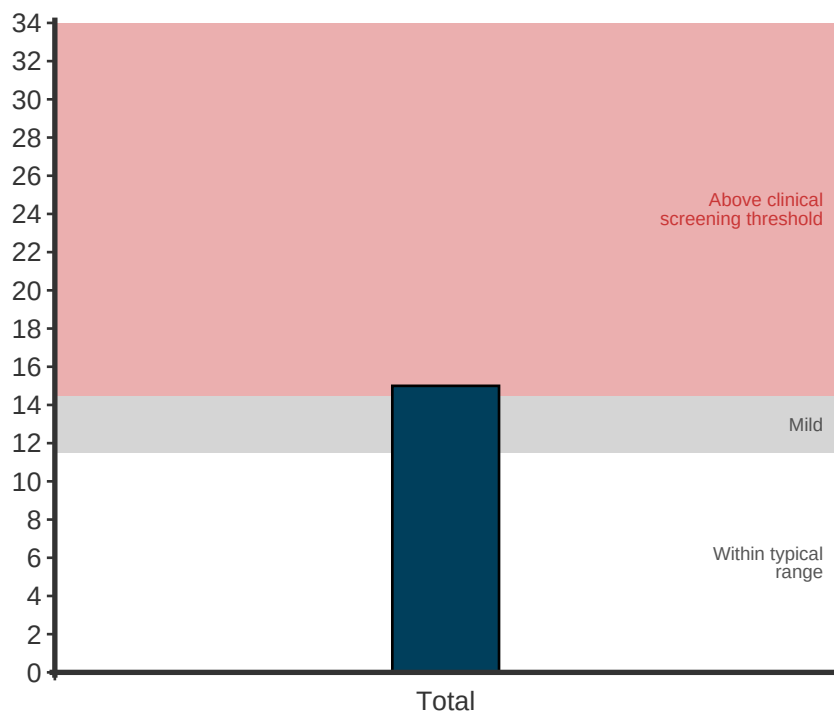
Informant Details

Informant's Name	Test
Informant's Relationship to Child	Father

PSC-17 Results

	Raw Score	Non-Clinical Percentile	Descriptor
Total (0-34)	15	93	Above clinical screening threshold
Internalising (0-10)	3	84	Mild
Attention (0-10)	9	99.5	Above clinical screening threshold
Externalising (0-14)	3	53	Within typical range

PSC-17 Total Score

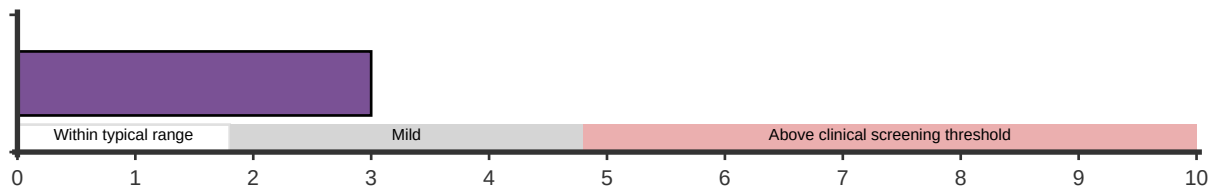




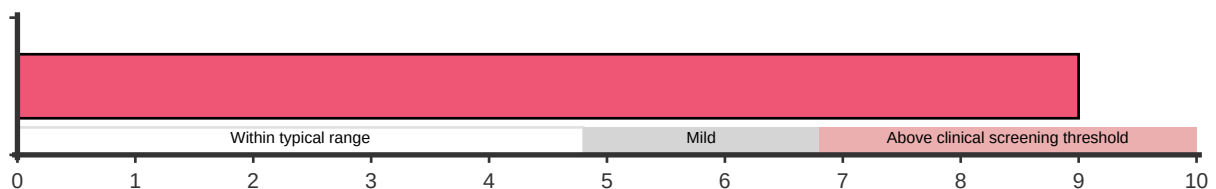
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PSC-17 Subscale Scores

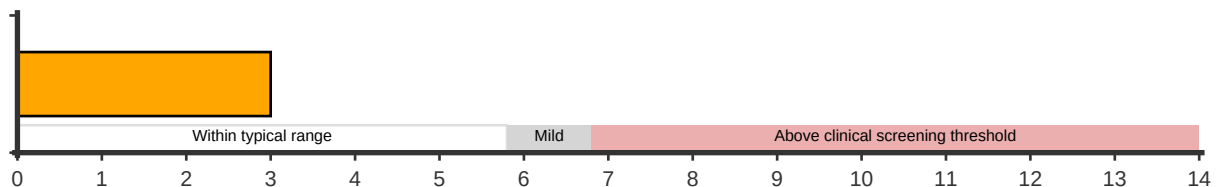
Internalising



Attention



Externalising



Interpretation

The Pediatric Symptom Checklist-17 (PSC-17) was administered on 03 February 2026, with a total score of 15 out of 34, which is at the 93rd percentile compared to a non-clinical community sample. This score is at the clinical cut-off of 15, suggesting elevated overall behavioural and emotional concerns.

Internalising. The internalising subscale score is 3 out of 10, which is at the 84th percentile. This score is below the clinical cut-off of 5 but in the mild range, suggesting mild internalising symptoms. In particular, the respondent endorsed the following items:

- 11. *Seems to be having less fun (Sometimes)*
- 9. *Is down on themselves (Sometimes)*
- 2. *Feels sad, unhappy (Sometimes)*

Attention. The attention subscale score is 9 out of 10, which is above the 99th percentile. This score is above the clinical cut-off of 7, suggesting elevated attention difficulties. In particular, the respondent endorsed the following items:



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7. *Has trouble concentrating (Often)*
3. *Daydreams too much (Often)*
1. *Fidgety, unable to sit still (Often)*

Externalising. The externalising subscale score is 3 out of 14, which is at the 53rd percentile. This score is below the clinical cut-off of 7, suggesting externalising symptoms within the typical range.

Scoring and Interpretation Information

For comprehensive information on the PSC-17, [see here](#).

The PSC-17 produces a raw Total score, three raw subscale scores, and corresponding cut-off classifications and percentiles. Higher scores indicate more parent-reported psychosocial concerns.

PSC-17 Scoring

The Total score is calculated by summing all 17 items (range: 0-34). Subscale scores are calculated by summing the items belonging to each subscale.

1. Internalising subscale (items 2, 6, 9, 11, 15; range: 0 to 10): assesses inner distress, including sadness, hopelessness, low self-worth, worry, and anhedonia
2. Attention subscale (items 1, 3, 7, 13, 17; range: 0 to 10): assesses attentional difficulties, including motor restlessness, daydreaming, concentration problems, and distractibility
3. Externalising subscale (items 4, 5, 8, 10, 12, 14, 16; range: 0 to 14): assesses outwardly directed behavioural difficulties, including fighting, not listening, not understanding others' feelings, teasing, blaming, refusing to share, and taking others' belongings

PSC-17 Severity Categories

A three-tier descriptor framework is applied to the Total score and to each subscale. The descriptor bands are anchored to two empirically derived cut-offs per scale: the primary and widely used screening cut-off (Gardner et al., 1999), and a more sensitive cut-off (Stoppelbein et al., 2012). The alternative cut-off marks the lower bound of a "Mild" subthreshold band that flags scores worth monitoring without indicating a positive screen.

Within typical range:

- Total: 0–11
- Internalising: 0–1
- Attention: 0–4
- Externalising: 0–5

Mild:

- Total: 12–14
- Internalising: 2–4
- Attention: 5–6
- Externalising: 6

Above clinical screening threshold:

- Total: 15 or higher
- Internalising: 5 or higher
- Attention: 7 or higher
- Externalising: 7 or higher



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A score above the clinical screening threshold indicates that further psychosocial assessment is warranted. It does not indicate a diagnosis. A score in the Mild range is intended as a monitoring flag rather than a definitive interpretation. It falls below the primary screening threshold but may be worth tracking across administrations and potentially warrant clinical attention. A score within the typical range does not rule out clinical concerns; clinicians should integrate the PSC-17 result with other clinical information when making decisions about further assessment.

PSC-17 Percentile Context

A community percentile is presented for the Total score and each subscale, expressing the respondent's score relative to a non-clinical community sample of children attending routine paediatric appointments (Stoppelbein et al., 2012). A percentile of 50 represents a typical level of parent-reported psychosocial concerns; a percentile of 90 indicates that the child's score is higher than 90% of the non-clinical community sample. Percentiles are presented as supporting context, with the cut-off classification serving as the primary interpretive framework.

PSC-17 Subscale Interpretation

Subscale-level interpretation indicates which domain of psychosocial concerns is most prominent in the parent's report. An elevation (a score above the clinical screening threshold) on each subscale suggests that, per parent-report, the child is experiencing:

1. Internalising: Inner distress relevant to anxiety, low mood, hopelessness, low self-worth, or anhedonia. Elevated internalising symptoms in this age group commonly co-occur with depressive and anxiety conditions, family-environment stressors, peer difficulties, and bullying.
2. Attention: Attentional and motor-activity concerns relevant to attention-deficit/hyperactivity disorder. The items do not distinguish between primary attention problems and attention difficulties secondary to other conditions; learning difficulties, anxiety, and sleep disturbance can produce overlapping presentations.
3. Externalising: Difficulties consistent with conduct or oppositional concerns. Behavioural presentations in this age group commonly vary across settings (home, school, after-school care) and are often influenced by family-environment factors.

Co-occurrence across multiple subscales is common in childhood and is associated with greater overall psychosocial complexity than single-domain elevations.

A subscale score in the Mild range falls below the primary screening threshold but warrants tracking across administrations: a reliable increase from typical to Mild, or from Mild to above the clinical screening threshold, may warrant additional clinical attention even when the absolute score remains modest.

PSC-17 Change Score Interpretation

When PSC-17 scores are available from multiple administrations, changes in scores can support progress monitoring during care. Change is reported as the raw score difference between the respondent's baseline and the current administration, for the Total score and each subscale. The interpretive text indicates the direction and magnitude of change in plain language and applies the published PSC-17 reliable-change thresholds (Murphy et al., 2016, for the Total; Kamin et al., 2015 and McCarthy et al., 2016, for the subscales).

NovoPsych reports two types of change:

1. Reliable change: The change exceeds the reliable change index (RCI) (6 points for the Total, 2 points for each subscale), indicating that the change is unlikely to reflect measurement error alone.
2. Clinically significant change: A reliable change that also crosses a validated clinical cut-off in



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either direction (Gardner et al., 1999), indicating a change in risk status (i.e., a transition into or out of the screening-positive range). Crossings into or out of the Mild range are tracked as monitoring signals but do not constitute clinically significant change.

PSC-17 Graphs

On a first administration of the PSC-17, four graphs are presented: one for the Total Score and one for each subscale (Internalising, Attention, Externalising)

The Total Score graph is a vertical bar showing the score against the full 0 to 34 range. The background is divided into three coloured zones (Within typical range = white, Mild = grey, Above the clinical screening threshold = red), so the descriptor band the score falls in is visible at a glance.

Beneath the Total Score, three horizontal subscale graphs are presented, each spanning the subscale's range (Internalising 0 to 10, Attention 0 to 10, Externalising 0 to 14). A coloured band under each bar shows the same three descriptor zones.

When two or more administrations are available, the single-administration graphs are replaced by trajectory graphs. These visualisations enable clinicians to track treatment progress, identify patterns of improvement or deterioration over time, and observe band transitions across administrations.

The Total Score trajectory graph shows the Total score across administration dates, with the same three coloured zones shaded as the chart background. Subscale Trajectory panel with three stacked line graphs (Internalising, Attention, Externalising), each with its own y-axis matching the subscale's score range and the same three shaded descriptor zones.

Client Responses

Internalising

		Never	Sometimes	Often
2	Feels sad, unhappy	0	1	2
6	Feels hopeless	0	1	2
9	Is down on themselves	0	1	2
11	Seems to be having less fun	0	1	2



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Client Responses (cont.)

		Never	Sometimes	Often
15	Worries a lot	0	1	2

Attention

		Never	Sometimes	Often
1	Fidgety, unable to sit still	0	1	2
3	Daydreams too much	0	1	2
7	Has trouble concentrating	0	1	2
13	Acts as if driven by a motor	0	1	2
17	Distracted easily	0	1	2

Externalising

		Never	Sometimes	Often
4	Refuses to share	0	1	2
5	Does not understand other people's feelings	0	1	2
8	Fights with other children	0	1	2
10	Blames others for their troubles	0	1	2
12	Does not listen to rules	0	1	2
14	Teases others	0	1	2
16	Takes things that do not belong to them	0	1	2